TE-04 Hyperthermia

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026

Heat exposure plus altered mental status is heat stroke until proven otherwise. Begin active cooling immediately; do not wait for a temperature or exclude heat stroke because the first temperature is below 104 F.

Recognition and immediate field flow

Step	Approved action
1. Remove from heat	Move to a cool area, remove tight or excess clothing, limit exertion, assess glucose, and obtain temperature when feasible without delaying care.
2. Classify	Cramps: painful spasms. Exhaustion: weakness, dizziness, nausea, or heavy sweating without CNS dysfunction. Heat stroke: heat exposure plus altered mental status, seizure, or coma.
3. Heat stroke?	Start active cooling concurrent with resuscitation. Ice- or cold-water immersion is preferred when safe. Use tepid-water immersion or tarp-assisted ice-water cooling when immersion is unavailable.
4. Cooling endpoint	Use rectal core temperature when feasible without interrupting care. Stop active cooling once core temperature is below 102 F (39 C).
5. Perfusion	For hypotension, dehydration, or poor perfusion, give isotonic crystalloid in reassessed aliquots: adult 500 mL; pediatric 20 mL/kg. Do not use a routine fixed maximum.
6. Transport	Continuously reassess airway, ECG, mental status, BP, perfusion, and core temperature. Notify the receiving facility early for suspected heat stroke.

Provider-level actions

Level	Approved actions
EMT	Move to a cool environment, remove excess clothing, check glucose, begin active cooling, and give oral fluid only for mild illness with intact mentation and swallowing.
AEMT	All EMT care plus IV/IO isotonic crystalloid for hypotension, dehydration, or poor perfusion with reassessment after each aliquot.
Paramedic	All prior care plus cardiac monitoring and protocol treatment of seizure, dysrhythmia, shock, or airway failure.

Critical safeguards

- Do not delay cooling to obtain a temperature. Heat exposure with altered mental status is heat stroke until proven otherwise.
- Antipyretics and dantrolene are not treatments for environmental or exertional heat stroke. Use active external cooling.
- Do not overcool. Stop active cooling once core temperature is below 102 F (39 C).

TE-04 | COOLING METHODS, FLUIDS, AND HANDOFF

Active cooling methods

Topic	Required action
Preferred	When safe and feasible, use whole-body neck-down cold-water or ice-water immersion with continuous access to the airway and patient monitoring.
Alternative	Use tarp-assisted ice-water cooling, tepid-water immersion, cold shower, ice sheets/towels, ice packs to high-flow areas, evaporative spray and fan cooling, or a combined approach when immersion is unavailable.
Monitoring	Track mental status, temperature, shivering, airway, respiratory effort, BP, pulse, perfusion, and ECG. Adjust cooling to avoid overshoot below the endpoint.
Concurrent care	Manage seizures, agitation, shock, hypoglycemia, dysrhythmia, and airway failure under the applicable approved pathway while cooling continues.

Fluids and disposition

Topic	Required action
IV/IO fluids	Use reassessed isotonic crystalloid aliquots only for hypotension, dehydration, or poor perfusion. Reassess lungs, work of breathing, BP, perfusion, and clinical response after each aliquot.
Oral fluids	Reserve oral fluids for mild illness when the patient is alert, has intact swallowing, and has no vomiting or altered mental status.
Destination	Suspected heat stroke requires emergency evaluation and early receiving-facility notification. Do not delay effective cooling for routine transport tasks when immersion is actively available.

Handoff and release note

- Report activity, environment, onset, duration, cooling methods/times, temperature trend, glucose, fluids, neurologic status, ECG findings, and response to care.

- Use the applicable adult or pediatric airway, shock, seizure, dysrhythmia, and altered-mental-status protocol as indicated.

- This direct source PDF replaces the prior imported sheet and matches Claiborne-approved TE-04 app content reconciled October 30, 2026.